

ZORAM MEDICAL COLLEGE (ZMC) & HOSPITAL

Department of Neurology & Behavioral Health

Falkawn, Aizawl, Mizoram 796005

CONFIDENTIAL CLINICAL MEDICAL RECORD - NEUROLOGY CONSULTATION

Patient Name: Sailo, Lalhmingmawii	MRN: ZMC-2026-3180	Sex: Female
DOB / Age: 18/08/1962 (Age: 64)	Date of Exam: 24/08/2026	Primary Caregiver: Lalrinzuala Sailo (Husband)
Attending Neurologist: Dr. Lalremruata Ralte, MD, DNB (Neurology)	Referring Physician: Dr. C. Lalduhawma, MD (Psychiatry)	Clinical Status: Outpatient Evaluation

REASON FOR EVALUATION

Evaluation of progressive behavioral alterations, disinhibition, loss of social empathy, and compulsive sweet consumption over 20 months.

CLINICAL HISTORY (HISTORY OF PRESENT ILLNESS)

The patient is a 64-year-old right-handed Mizo female, former church choir director and civil services employee in Aizawl, presenting with insidious personality changes. Over the past 20 months, she has exhibited progressive behavioral disinhibition, making tactless remarks to neighbors, compulsive hoarding of sweets and biscuits (hyperorality), and profound emotional blunting toward family bereavement. Functional Assessment: Dependent on husband for financial management (made impulsive, erratic online and local purchases), cooking (leaves stoves unattended and ignores recipes), and complex household organization. She can independently dress and bathe but shows neglect in personal hygiene unless prompted. Refuses to drive. Behavioral Symptoms: Severe apathy alternating with impulsive disinhibition, stereotypic clapping movements, social withdrawal from church community, and altered sleep-wake cycle.

PAST MEDICAL HISTORY	ACTIVE MEDICATIONS
- Mild Hypertension - No prior psychiatric history, head trauma, or alcohol use	- Telmisartan 20 mg PO daily - Multivitamin tablet daily

PHYSICAL & NEUROLOGICAL EXAMINATION

Alert, restless, exhibits utilization behavior (frequently grabbing objects from physician's desk), inappropriate jocularity (Witzelsucht). Cranial nerves intact. Motor: Normal power 5/5, no rigidity or tremor. Primitive reflexes: Positive grasp and palmomental reflexes bilaterally. Reflexes: 2+ symmetric. Gait: Normal pace, restless wanderer.

STANDARDIZED COGNITIVE TESTING

Mini-Mental State Examination (MMSE): Score 25/30 | Frontal Assessment Battery (FAB): Score 7/18 (Severe Frontal Executive Dysfunction)

- Orientation: 10/10 (Preserved orientation to time and place)
- Registration: 3/3
- Attention & Calculation: 4/5
- Recall: 3/3 (Episodic memory completely preserved)
- Language & Visual Construction: 5/9 (Clock Drawing structurally correct; severe failure on verbal fluency: Letter 'F' fluency = 3 words; marked perseveration on motor sequencing and go/no-go tasks)

DIAGNOSTIC WORKUP & NEUROIMAGING BIOMARKERS

- Laboratory: Normal thyroid, B12, metabolic panels, autoimmune encephalitis panel negative.
- Brain MRI: Marked asymmetric bilateral frontal lobe and anterior temporal pole atrophy ('knife-edge' gyral thinning) with anterior horn ventricular enlargement. Hippocampal structures relatively preserved (MTA Score: Grade 1, Frontal Atrophy Grade 3). Fazekas Grade 0.

DIAGNOSTIC IMPRESSION

- Major Neurocognitive Disorder due to Behavioral Variant Frontotemporal Dementia (bvFTD) (ICD-10: G31.09 / F02.80).
- Diagnosis supported by early behavioral disinhibition, loss of empathy, hyperorality, executive dysexecutive syndrome with relative preservation of episodic memory and visuospatial skills.

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

- Behavioral & Environmental Management: Structured daily routine, secure kitchen cupboards to control sweet consumption, supervision for financial assets.
- Pharmacotherapy: Initiate Sertraline 25 mg PO morning, titrate to 50 mg after 2 weeks for compulsive behaviors and disinhibition. Avoid cholinesterase inhibitors (may worsen behavioral agitation in FTD).
- Caregiver Respite: Comprehensive caregiver psychoeducation on frontotemporal neurodegeneration.
- Follow-Up: Clinic follow-up in 8 weeks to assess behavioral stabilization.

Electronically Verified & Signed by:

Dr. Lalremruata Ralte, MD, DNB (Neurology)
Consultant Neurologist | Reg No: MIZ-09412
Zoram Medical College & Hospital, Falkawn, Aizawl

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